

HospitalOS

Hospital Operating System for Africa

Tenant Guide

Hospital staff reference · Version 1.0 · July 2026 · AgoroX Technologies

1. Who this guide is for

This guide is for hospital staff using HospitalOS day to day — clinicians, diagnostic and pharmacy staff, finance and administration teams. It covers every module in the system and, where it matters, why a screen behaves the way it does.

Role	Scope	Account
Administrator	Your hospital's full HospitalOS deployment	Any account with the Super Admin role
Clinical staff	Patient care, diagnostics, pharmacy for your role	Doctor / Nurse / Lab Scientist / Radiographer / Pharmacist accounts
Finance staff	Billing, payments, claims	Cashier account

Two-level permissions.

Every role has an AREA list (which sidebar groups it can open) and a GRANTS list (which specific actions it can take once inside). Section 3 covers this in full, with the actual permission table.

2. Signing in and how the system is organised

Sign in with your hospital email. An administrator sets your role under Administration → Users & roles.

2.1 The ten sidebar groups

The sidebar is arranged by workflow, roughly in the order work happens through a hospital, not by department hierarchy:

Group	What lives there
Overview	Dashboard, alerts, worklist, communication hub
Patient care	Registration/ADT, wards, ICU, emergency, maternity, records, renal, bookings, referrals
Diagnostics	Laboratory, blood bank, imaging, radiotherapy, instruments gateway
Pharmacy	Dispensing, inventory, formulary
Finance & trade	Billing, payments, claims, procurement, stores
Operations	Scheduling, CSSD, biomedical, facility, fleet, support, visitors
Academic	Training, logbooks, CME, research, ethics committee
Public health	Surveillance, immunisation, outreach, reporting
Intelligence	Analytics, reports, forecasting
Administration	Users & roles, facilities, instruments gateway, pricing, documents, audit, settings

You only see what your role grants.

A pharmacist's sidebar shows Overview and Pharmacy; a cashier's shows Overview and Finance. This is not secrecy — it keeps each person's view to what they actually use.

3. Roles and permissions

Area permissions gate the sidebar and routes. Action permissions gate the buttons within an area. Separation of duties is enforced, not just suggested:

Role	Can	Cannot
Nurse	Admit patients, record vitals, file clinical notes	Discharge a patient
Lab Scientist	Collect samples, enter results, verify results	Reach Patient care at all
Radiographer	Order and report imaging studies	Verify laboratory results
Pharmacist	Dispense medication, restock inventory	Reach Diagnostics at all
Cashier	Take payments, file insurance claims	Approve a claim
Records Officer	Register new patients	Admit or discharge a patient
Doctor	Full patient-care and diagnostics actions	Administration functions
Super Admin	Everything	—

The last-admin protection.

The last active Super Admin account cannot be deactivated by anyone, including another Super Admin — this prevents a hospital from accidentally locking itself out of its own system.

4. Overview and the alert system

The Dashboard opens on live occupancy, admissions, and revenue charts. Every stat card links directly into the module behind it.

4.1 Alerts — eleven sources, one feed

Source	Raises an alert when...	Severity
Laboratory	A result crosses a panic threshold	Critical
Critical care	Any ICU/HDU vital goes critical	Critical
Radiology	A report is flagged with an urgent finding	Critical
Maternity	A newborn's Apgar score is below 7	Critical
Blood bank	A group is below reorder / a unit nears expiry	Critical / Warning
Pharmacy	A drug is out of stock / below reorder	Critical / Warning
Instruments	A device errors or goes offline	Critical / Warning
Renal	A scheduled dialysis session is overdue	Critical
Operations	Equipment under repair / a vehicle is out of service	Warning
Oncology	A chemotherapy cycle is overdue	Warning
Public health	A notifiable disease is trending upward	Warning

Most alerts clear themselves — they read live state, so fixing the underlying problem clears the alert automatically, nothing to dismiss.

4.2 Notifications

Not yet functional as email/SMS push.

The bell icon currently surfaces booking requests inside the app only. It does not yet push a notification outside

HospitalOS — that requires the Communication hub's channels to be connected to a real SMS/email gateway. Its count does not shrink on its own; a booking only leaves the bell once someone actually confirms or declines it.

5. Patient care

5.1 Registration, admission and the ward board

The bed board and ADT read the same registry, so a bed taken in one is unavailable in the other instantly — double-booking is not possible.

5.2 Accommodation tiers

Tier	Rate / night
General Ward	■15,000
Semi-Private	■35,000
Private Room	■60,000
Private Suite	■120,000
VIP Suite	■220,000
Executive Suite	■350,000
Critical Care	■180,000

Catalogue defaults — every rate can be overridden for your hospital. See Section 9.

5.3 The medical record

SOAP-structured clinical notes, an ICD-10 problem list, and recorded allergies. Filed notes cannot be edited or deleted — a correction is filed as an amendment referencing the original, with both remaining visible.

Allergy safety is enforced, not advisory.

A severe allergy on a patient's record disables the Pharmacy dispense button outright for that drug — there is no override in the interface.

5.4 Referrals

Referrals in and out of your facility are tracked from first contact through to acceptance or decline, and through to the patient's arrival. See Section 5.5 below for the full workflow.

5.5 Renal & dialysis, ICU/HDU, Emergency, Maternity, Online bookings

- **Renal & dialysis** runs a haemodialysis programme (vascular access, schedule, per-session fluid removal) and a CKD registry that auto-stages from eGFR.
- **ICU/HDU** tracks five vitals per bed with panic thresholds; a critical reading marks the patient Unstable and raises an alert automatically.
- **Emergency** sorts by triage acuity first, arrival time second.
- **Maternity** tracks labour through to delivery; a newborn Apgar below 7 raises a critical alert immediately.
- **Online bookings** holds appointment requests from your website. Confirming and checking in a booking calls the same function Outpatient uses internally — a checked-in booking is a genuine addition to today's queue, not a separate list.

6. Diagnostics

41 laboratory tests across eight disciplines, and 27 imaging protocols across Ultrasound, CT, MRI, General Radiography, and Mammography.

6.1 Laboratory

Orders move Ordered → Collected → Resulted → Verified; each result flags Low/High/Critical live against its reference range.

6.2 Instruments & devices gateway

One interoperability hub, reachable from both Diagnostics and Administration:

Category	Protocol	Examples
Laboratory analyzer	HL7 v2 / MLLP	Sysmex, Cobas, a 2009 analyzer still in service
Imaging modality	DICOM 3.0	CT, MRI, ultrasound, a 2007 CR reader
Radiotherapy system	DICOM-RT / proprietary	A modern LINAC and a manual Cobalt-60 unit
Printer	IPP / raw socket / serial	Label, report, and legacy dot-matrix printers

Not yet functional as live network listening.

Device management and monitoring are real. The network listener itself — an MLLP socket, a DICOM SCP, a print daemon — is server territory a browser cannot open; every action in this screen calls the exact function a live listener would call, so nothing downstream needs to change when the real listener is built.

6.3 Biobanking & lab utilities

Biobanking tracks long-term specimen storage across four capacity-limited units. Lab utilities holds clinical calculators, unit converters, and bench reference material.

7. Pharmacy

Dispensing checks recorded allergies before completing a transaction and blocks any quantity greater than available stock. Inventory alerts clear themselves once stock is restocked above the reorder level.

8. Finance & trade

Billing aggregates charges from five sources automatically: laboratory, pharmacy, radiology, theatre, and accommodation. A theatre case becomes billable the moment it enters theatre, not when scheduled. Claims move Submitted → Approved/Rejected → Paid — a cashier can file a claim but cannot approve it.

9. Making HospitalOS your own

9.1 Setting your own prices

Administration → Pricing lets you override the catalogue default for any test, drug, imaging study, procedure, or accommodation tier. Every billing calculation and every price shown to staff reads through the same value.

Past charges are never rewritten.

A patient billed before you changed a price keeps their original total; only new transactions use the new price.

9.2 Your branding

Administration → Settings has a hospital name and logo URL field. Both appear top-right on every screen, live, within seconds of saving.

9.3 Your documents



Administration → Documents & templates accepts real uploads — drag a file in, assign any category you like, and it is immediately downloadable, renameable, and deletable.

Not yet functional as permanent storage.

Uploaded files live in your browser session only until server-side storage is connected — they do not survive a page reload.

10. The audit trail

Administration → Security & audit is append-only by construction: entries are frozen the moment they are written, with no update or delete API at all. Each entry is hash-chained to the one before it, so editing a past record breaks verification at that record, and deleting one breaks verification at the next.

Security note.

This is tamper-evident, not tamper-proof — a browser cannot stop someone with developer tools. Its value is architectural: the identical chain design carries over unchanged to server-side storage, where it becomes a genuine, enforceable control.

11. Getting help

Press Ctrl+K (Cmd+K on Mac) from anywhere to search screens, patients, and the full help library at once. Help & documentation is pinned at the bottom of the sidebar, outside the workflow groups, reflecting that it is a standalone reference.

11.1 Downloading this guide

This guide is available at any time from Help & documentation — click “Download the full Tenant Guide” on the landing page.

12. Known limitations checklist

An honest account. Review before entering anything you cannot afford to lose.

- Data does not persist — every module runs in-memory for this preview; records reset on reload.
- Sign-in is not yet server-verified — credentials are checked in the browser.
- The audit trail is tamper-evident, not tamper-proof, until it moves server-side.
- The instruments gateway simulates live device listening; management and monitoring are real.
- Uploaded documents do not persist across a page reload.

Important.

Do not enter real patient data into this build. It is a working preview of a production system, not production itself.

Questions about this guide?

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